Session 8 (PUBLISH): Exercise sheet

Foundations of Clinical Research · Level 1 · Neudata · *#ClearDataClearImpact* · Week 8 · Saturday 19 December 2026 · online, 19:30–22:30

Name: _____ **Department:** _____

Type your answers into this Word file or print the PDF.

Part	Exercise	When	How
A	Writing exercises: fix the spin, verb ladder, results sentence, title from PICO	homework	Alone
B	**Manuscript workshop 2**: improve your Results section	20:05 (25 min)	17 min whole class on Q1, then 8 min on your own draft and title. Send it by 23:59 on Sunday 20 December
C	Getting published: spot the predatory journal; responding to reviewers	homework	Alone
D	**Answer grid: "Be the reviewer"**	21:05 (25 min)	Whole class with polls; you fill in your own grid

Files you need (sent by email): the original PDFs of Paper 1 (Boulet 2024) and Paper 2 (Hyun 2023), the class Q1 Results from Session 7 (PDF), your own Results draft and your results pack.

Part A: Writing exercises (homework)

Practice for after the session.

A1. Fix the spin

Rewrite each sentence so it says only what the data and the design allow.

#	Original sentence (from the papers)	Your rewrite
a	Paper 1 (p.10), primary outcome p = 0.42, difference 13.2 ml/kg (95% CI −15.2 to 41.6): the authors base the case for a larger trial on "the non-significant trend in fluid balance reduction".	
b	Paper 2 (p.4): "a positive fluid balance on the first ICU day did not impact the 28-day mortality".	
c	Paper 2 (title): "Impact of a cumulative positive fluid balance during the first three ICU days in patients with sepsis …"	
d	Paper 1 (p.10): the protocol obtained "the lowest fluid balance on Day 5 ever published".	

A2. Climb the verb ladder

Choose the strongest verb each design allows: **was observed in · is associated with · may contribute to · reduced / did not reduce · causes**.

#	Study	Strongest honest verb
a	A case series of 12 ICU patients with septic shock who developed pulmonary oedema after large fluid volumes	
b	A cohort of 3,000 septic patients: positive fluid balance and mortality, adjusted for severity	
c	A single randomised trial of 48 patients: restrictive protocol and day-5 fluid balance	
d	Five large randomised trials and a meta-analysis, all consistent	
e	A cross-sectional survey of how completely fluid-balance charts are filled in on two ICUs	

A3. Write the results sentence (number + CI, no adjectives)

Write one sentence for each, in the style "In [whom], [outcome] was [effect] ([measure] [value], 95% CI [lower] to [upper])".

1. Paper 2, Day-3 cohort: 28-day mortality, positive vs negative fluid balance, HR 1.56, 95% CI 1.10 to 2.22, P = 0.012.
2. Paper 1: death by day 28, 6/24 (restrictive) vs 6/24 (control), relative risk 1.0, 95% CI 0.38 to 2.66.
3. Paper 2, Day-1 cohort: HR 1.17, 95% CI 0.85 to 1.60, P = 0.354.

A4. A title from your PICO

Formula: **[Design] of [Intervention / exposure] vs [Comparator] in [Population]: effect on / association with [Outcome]**. 10–16 words, design stated, no "impact", "novel", "first".

My PICO: P _____ I/E _____ C _____ O _____

Working title: _____

Word count: ____ Design named? ☐ Calibrated verb (association for observational designs)? ☐

Part B: Manuscript workshop 2: improve your Results section (25 minutes, 20:05)

How the 25 minutes are used

Clock	Who	Task
20:05–20:10	Whole class, Q1	**Numbers**: every number in the class Q1 Results against results pack Q1
20:10–20:14	Whole class	**Tables and figure**: title, units, n, abbreviations. Does each stand alone?
20:14–20:19	Whole class	**Words**: no interpretation, no "trend", no causal verbs
20:19–20:22	Whole class	**Working title**: design stated, "associated"
20:22–20:30	You	Run the same checks on your own draft. Write your working title and paste it in the chat
By 23:59, Sunday 20 December	You	Final check, then email your Results section

The improved class Q1 Results are sent as a PDF after the session.

Final check before you send

- ☐ Every number agrees with your results pack
- ☐ Percentages with denominators, estimates with 95% CIs
- ☐ Crude and adjusted estimates both reported
- ☐ Table 1 and one main table or figure that stand alone
- ☐ No interpretation and no causal words
- ☐ Working title and AI-use statement

Other sections (taught in 8.1, optional to submit)

If you have time, add any of these in the same file. Trainers give feedback on them.

- **Introduction:** three paragraphs: known, not known, aim
- **Discussion:** answer, compare (e.g. with Paper 2), explain, limitations with their direction, implications, conclusion
- **Abstract:** structured, 250 words or fewer, numbers identical to your Results

Deliverable

`Surname_Firstname_Results_QN.docx`, emailed to your reviewer and to the instructor (myluong1710@gmail.com) **by 23:59 on Sunday 20 December 2026 (Vietnam time)**. Then the peer review (`Peer_Review_Instructions.pdf`): your review is due on Wednesday 30 December 2026, and your revised Results section and response letter (`Response_to_Reviewers_Template.docx`) on Saturday 9 January 2027.

Part C: Getting published (homework)

C1. Spot the predatory journal

1. Find the red flags in this (invented) invitation. How many can you find?

> **From:** editorial.board.gjcmr@freemail.com
>
> **Subject:** Invitation to publish in our esteemed journal!!
>
> Dear Respected Professor/Doctor,
>
> Greetings! We have gone through your excellent article and are delighted to invite you to submit to the *Global Journal of Clinical Medicine, Nursing, Engineering and Management Research*. We publish all areas of science.
>
> Our journal has an Impact Factor of 9.2 (International Universal Index). We guarantee peer review within 48 hours and publication in 5 days. Processing charges will be communicated after acceptance. Special discount this week only!
>
> Kindly submit your manuscript by replying to this e-mail.

2. Which journal would you trust? For each (invented) profile, write *Trust / Check further / Avoid* and one reason.

Journal profile	Your verdict	Reason
J1: Indexed in MEDLINE (confirmed in the NLM Catalog), editor names verifiable at universities, fee (APC) shown on the website, median 9 weeks to first decision, uses CONSORT/STROBE checklists.		
J2: Says "indexed in Google Scholar and Index Copernicus", promises acceptance in 72 hours, no fee information, editorial board photos without affiliations.		
J3: Open access, listed in DOAJ, small society journal, no impact factor yet, clear peer-review and retraction policy.		
J4: Its name differs by one word from a well-known journal; the website address is different from the real journal's; it e-mailed you first.		

3. Three checks you can always do: Think.Check.Submit checklist · DOAJ (Directory of Open Access Journals) · NLM Catalog ("currently indexed in MEDLINE"). You can also ask your librarian.

C2. Responding to reviewers

Imagine your own proposed study has been completed and submitted. Write a response row for each comment (thank, answer, say exactly what changed). One of them deserves a polite rebuttal.

#	Reviewer comment	Our response	Change made (page, line)
R1.1	"The title says your intervention *improves* outcomes, but this is an observational study."		
R1.2	"Please report the main result with a confidence interval, not only the p-value."		
R1.3	"Text and Table 2 give different numbers of patients analysed (212 vs 208)."		
R2.1	"Please add 1-year mortality." (Your protocol followed patients to day 28 only.)		

Tone checklist: ☐ thanked the reviewer ☐ answered every point ☐ said where the manuscript changed ☐ disagreed only with evidence, politely

Part D: Answer grid: "Be the reviewer" (25 minutes, 21:05)

What you are doing

Two published papers answer our running question (**fluids in septic shock**) with different designs:

	Paper	Design
Paper 1	Boulet N, et al. *Crit Care* 2024;28:429 (restrictive strategy targeting non-resuscitative fluids, OPTIFLUID)	Randomised controlled pilot trial, 48 patients analysed
Paper 2	Hyun D-G, et al. *Ann Intensive Care* 2023;13:105 (cumulative positive fluid balance in the first three ICU days)	Propensity-score-matched cohort from a sepsis registry, 2516 patients

The whole class appraises **both** papers together. The instructor shows the right page on the shared screen, you vote **Yes / No / Can't tell** in a poll, and a volunteer gives the page and the sentence in the chat. Fill in your own grid as we go. "Can't tell" is a good answer when the paper does not say.

> **Page numbers** are the journal's own page numbers printed at the top of each page ("Page X of 13" for Paper 1, "Page X of 9" for Paper 2). Use the original (unannotated) PDF.

How the 25 minutes are used

Clock	What happens
21:05–21:08	The two papers and the 10 questions (slides).
21:08–21:23	Whole-class appraisal with polls. Paper 1: questions 4, 5, 7, 8 and 9. Paper 2: questions 4, 5, 8 and 9.
21:23–21:30	What may each design claim? The side-by-side summary.
At home	Answer the remaining questions (1–3, 6 and 10) for both papers.

Where to look

Question	Paper 1 (RCT): pages	Paper 2 (cohort): pages
1 Question	Abstract (p.1); last paragraph of Background (p.3)	Abstract (p.1); end of Background (p.2)
2 Design	Title (p.1); "Trial design and oversight" (p.3)	Title (p.1); Methods (pp.2–3)
3 Participants	"Patients" (p.3); flow chart Fig. 1 (p.5); Discussion limitations (p.10)	"Data source and patient selection" (p.2); Results first paragraph (p.3)
4 Comparison groups	"Randomization and blinding" (p.3); Table 1 (p.6)	"Propensity score matching" (pp.2–3); Table 1 (pp.5–6); text p.4
5 Bias	"Randomization and blinding" (p.3); Discussion (p.8)	Methods (pp.2–3); Discussion limitations (pp.7–8)
6 Outcome	"Outcomes" (p.4)	"Propensity score matching and outcomes" (p.3)
7 Numbers	"Primary outcome" (p.6); Table 3 (p.10)	Abstract (p.1); "Outcomes" (p.4)
8 Words vs design	Abstract conclusion (p.1); Discussion (pp.8, 10); Conclusions (p.11)	Title (p.1); Abstract conclusion (p.1); Discussion (pp.4, 7); Conclusions (p.8)
9 Limitations	Discussion (pp.8–10)	Discussion (pp.7–8)
10 Use	Your judgement (whole paper)	Your judgement (whole paper)

Answer grid (one copy per paper)

Paper: 1 / 2

#	Question (simplified appraisal checklist)	Yes	No	Can't tell	Page	Notes (one line)
1	**Question**: Can you write the study's PICO in one line?	☐	☐	☐		P: … I/E: … C: … O: …
2	**Design**: What is the design, and is it the right one for the question?	☐	☐	☐		
3	**Participants**: Who was included, and who was left out? Would your patients have been included?	☐	☐	☐		
4	**Comparison groups**: Were the groups similar at the start (Table 1)? How was this achieved (randomisation / matching / adjustment)?	☐	☐	☐		
5	**Bias**: Who knew which group patients were in (blinding)? Could that change the outcome?	☐	☐	☐		
6	**Outcome**: Was there ONE pre-specified primary outcome, measured the same way in both groups?	☐	☐	☐		
7	**Numbers**: What is the main result as an effect size with 95% CI? Is the CI narrow or wide?	☐	☐	☐		Effect: …… 95% CI: ……
8	**Words vs design**: Do the conclusions match the design ("associated" vs "caused"; no "trend" for p = 0.42)?	☐	☐	☐		Words found:
9	**Limitations**: Are the main limitations stated honestly? Name one the authors missed.	☐	☐	☐		Missed:
10	**Use**: Would this change practice in your unit? Why / why not?	☐	☐	☐		

My verdict in one sentence: _____

One thing I would copy into my own proposal: _____

Hints if you are stuck

- **Q1:** the PICO is usually in the last paragraph of the Introduction ("we aimed to …"). Is the paper's outcome the same as in our running question (28-day mortality)?
- **Q4:** a randomised trial makes groups similar by chance; a cohort can only match or adjust for what was measured. Find the sentence that says which.
- **Q5:** who decided how much fluid each patient received, and did they know the group?
- **Q7:** look for "difference" or "HR" followed by brackets. Does the interval include 0 (for a difference) or 1 (for a ratio)?
- **Q8:** search for the words *impact, show, trend, ever, regardless*.